

Q I've heard I need to exercise my pelvic floor. Where is my pelvic floor and why is it important to my pregnancy?

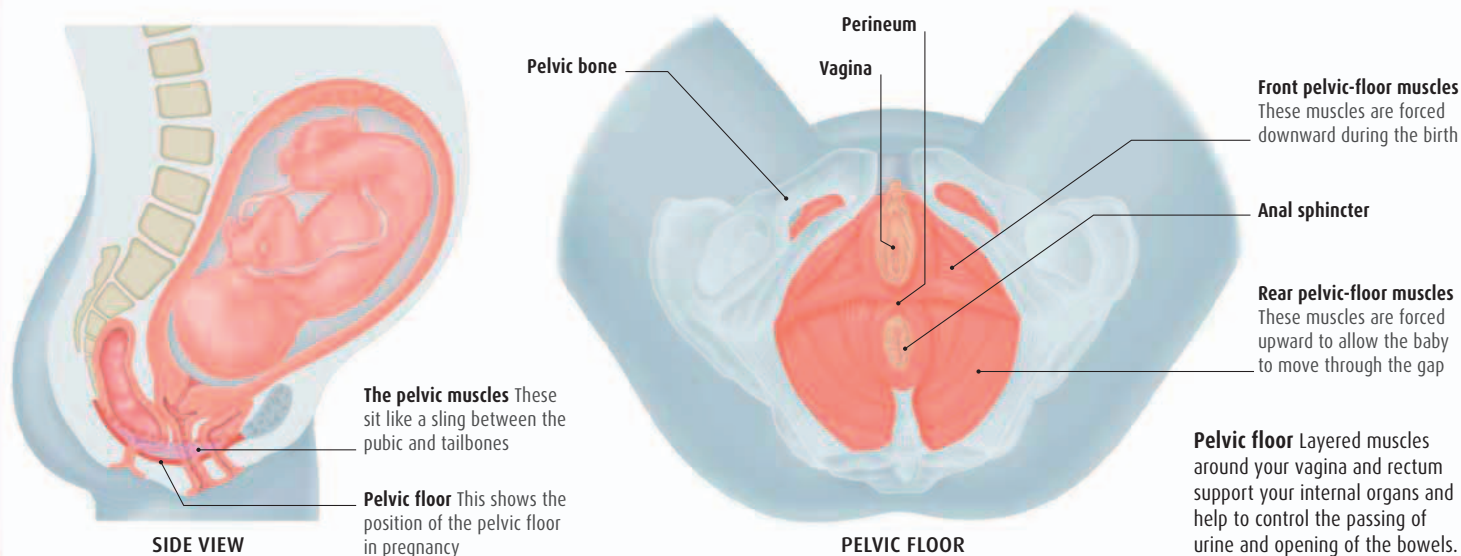
Your pelvic floor is a group of strong, layered muscles that stretches like a double sling, or a hammock, from your pubic bone to your tailbone. This sling supports your abdomen and holds in your reproductive and pelvic organs. During pregnancy, the muscles support the growing weight of your uterus and baby; during labor, they help guide your baby out of your body.

Some muscles of your body, such as your heart, are involuntary, but most are voluntary, which means you can consciously tighten and release them. You do this with your pelvic-floor muscles when you “hold in” if you need to go to the bathroom (the double sling of muscle has holes in it that correspond to your urethra, rectum, and vagina).

How the pelvic-floor muscles work

During pregnancy, your pelvic-floor muscles are loosened by the hormone relaxin, and then stretched and weakened by the weight of your growing baby bearing down on them. Having strong pelvic-floor

muscles is crucial: an already weak pelvic floor can mean that during pregnancy and afterward you can leak urine when you cough, sneeze, laugh, jump, or run—a condition known as stress incontinence. Also, during labor your pelvic floor forces your baby to turn in the cavity of your pelvis. He is then able to fit into the birth canal so you can push him out during second-stage labor. A weak pelvic floor may make this process less efficient. A surge of relaxin loosens your pelvic floor further to enable the baby to move through the birth canal: the pelvic-floor muscles near the front of your body are forced downward and those near the back are forced upward, creating an opening for your baby.



Q Are the pelvic-floor muscles permanently stretched after labor? What's a prolapse?

After labor, your pelvic-floor muscles should “ping” back into place relatively quickly, keeping your organs neatly tucked in as they were before. If the muscles are damaged during labor, however, you may suffer a “prolapse,” when your abdominal organs bear down into your vagina. Often, it's not until later in life that any damage becomes apparent and a prolapse occurs. So it's important to practice Kegel (pelvic floor) exercises throughout pregnancy and afterward, trying to reverse the damage and give the muscles back as much strength as possible.

Q I have a toddler who still expects to be carried. Will I harm the baby—or myself—if I lift him?

As long as your doctor hasn't told you not to, careful lifting is fine—it poses no threat to the safety of your unborn baby. Your back and pelvic floor, though, can be a different matter. It's vital that you protect your back by lifting carefully, whether you are picking up your toddler or lifting and carrying grocery bags or heavy objects.

To lift safely, plant your feet shoulder-width apart and flat on the floor. Bend down from your knees (not your waist), tucking your buttocks under and keeping your back straight.

Wrap your arms around your toddler, asking him to lock on with his legs, or hold the grocery bags or heavy object firmly. Push up with your thighs (use one arm to hoist yourself up against something if you need to), rather than straining your back. Once you're standing, adjust your toddler's position so it's comfortable for you—his legs straddling your belly, for example, or on one or other of your hips (if you carry your toddler on the side, switch sides each time to balance out the strain). If you carry several shopping bags, make sure that you distribute the weight evenly between both hands. Lifting also strains your pelvic floor, so always engage your pelvic-floor muscles before lifting anything, and practice your Kegel exercises frequently (see opposite).